

RADHAKISHAN HOSPITAL, KURUKSHETRA



AFFIX STICKER

Discharge Summary

RADHAKISHAN HOSPITAL, KURUKSHETRA
PICU DISCHARGE SUMMARY (Final Revised Version)
NABH HCO 6th Edition Compliant
Date of Discharge: 30-Oct-2025 | 4:25 PM
Admitted under: Dr. Swati Singhal & Dr. Lokender Goyal
Unit: Pediatric Intensive Care Unit (PICU)

1. PATIENT DEMOGRAPHIC PROFILE

Field	Details
UHID	25260004778
IP No.	25260001399
Patient Name	Master Mankirat Kumar
Age / Sex	2 Years / Male
Weight at Admission (kg)	14 kg
Weight at Discharge (kg)	14 kg
Date of Admission	22-Oct-2025
Date of Discharge	30-Oct-2025
Address	VPO Sirsala, District Kurukshetra
Contact Number	9896214598
Type of Discharge	Routine – Recovered & Stable

2. PROVISIONAL DIAGNOSIS (At Admission)

Severe right-sided pneumonia with sepsis and generalized tonic-clonic seizures.

3. FINAL DIAGNOSIS (With ICD-10 Code)

- J18.9 – Pneumonia, unspecified organism (right-sided, resolving)
- A41.9 – Sepsis, unspecified (resolved)
- R56.8 – Other and unspecified convulsions (post-infectious seizure disorder)

4. REASON FOR PICU ADMISSION

Acute respiratory distress and seizure due to severe pneumonia with sepsis requiring ventilator support and close hemodynamic monitoring.

5. SIGNIFICANT CLINICAL FINDINGS & HISTORY



Discharge Summary

Component	Findings
Initial General & Systemic Examination	Febrile, tachypneic, bilateral chest crepts (right > left), GCS 12/15, active on admission.
Significant Past History	None. Immunizations up to date.
Family History	No seizure or tuberculosis history.
Personal History	Full-term normal delivery, developmental milestones normal.
Review During Hospital Stay	Fever subsided; cough improved; no further seizures after levetiracetam initiation; extubated successfully on 25-Oct-2025; stable since.

6. COURSE DURING PICU STAY

A. Narrative Summary:

- Managed for severe right-sided pneumonia with sepsis and GTCS.
- Required ventilator support from 22–25 Oct 2025.
- Extubated successfully and maintained oxygen saturation >97% on room air.
- EEG: Abnormal with generalized dysrhythmia. MRI Brain: Normal.
- Treated with IV antibiotics, steroids, and anticonvulsant therapy.
- Cough improving, afebrile, and seizure-free at discharge.

B. Chronological Timeline

Date	Event / Intervention
22-Oct	Admitted with fever, pneumonia, and seizures; intubated and started on antibiotics.
25-Oct	Extubated; started on Levetiracetam.
27–28 Oct	Infection markers normalized (CRP 2 mg/L, PCT 0.04 µg/L).
28-Oct	MRI Brain normal; EEG abnormal (dysrhythmia).
29-Oct	Stable, afebrile; discharge plan initiated.
30-Oct	Improved cough; no seizure; discharged home.

7. PROCEDURES PERFORMED

- Endotracheal intubation and mechanical ventilation (22–25 Oct 2025)
- Extubation (25 Oct 2025)
- EEG (28 Oct 2025)
- MRI Brain (28 Oct 2025)

8. MEDICATIONS ADMINISTERED DURING PICU STAY

DRUG (GENERIC)	DOSE	ROUTE	START DATE	END DATE
LEVOFLOXACIN	140 mg	IV	22-Oct	27-Oct

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DRUG (GENERIC)	DOSE	ROUTE	START DATE	END DATE
LINEZOLID	140 mg	IV	22-Oct	27-Oct
AZITHROMYCIN	140 mg	IV	23-Oct	27-Oct
DOXYCYCLINE	35 mg	IV	23-Oct	27-Oct
LEVETIRACETAM	200 mg	IV	25-Oct	27-Oct
METHYLPREDNISOLONE	14 mg	IV	23-Oct	27-Oct
MAGNESIUM SULPHATE	1.4 mL	IV	23-Oct	27-Oct
PANTOPRAZOLE	15 mg	IV	23-Oct	27-Oct
FENTANYL / DEXMEDETOMIDINE	Infusion	IV	22-Oct	25-Oct
PARACETAMOL	15 mg/kg	IV	22-Oct	29-Oct (stopped)

☒ KPIs Captured:

Antibiotic days – 6 | Ventilation days – 3 | Sedation – 3 | Device days – 8

9. CONDITION AT THE TIME OF DISCHARGE

Parameter	Status
Sensorium	Alert, active, playful
Feeding	Adequate
Oxygen/Ventilation	Room air, SpO ₂ 99%
Neurological Status	Seizure-free, alert
Hemodynamics	Stable
Residual Deficit	None

☒ KPI: Stable and seizure-free at discharge.

10. MEDICATIONS PRESCRIBED AT DISCHARGE (Final Updated List)

DRUG NAME (GENERIC, CAPS)	DOSE	ROUTE	FREQUENCY	DURATION	INSTRUCTIONS
Syrup FEROPENEM	150 mg	Oral	Every 8 hours	6 days	After meals
Capsule DOXYCYCLINE (1/3 capsule)	2.5 mg/kg/dose	Oral	Every 12 hours	7 days	Mix with water; complete full course
Syrup AZITHROMYCIN	150 mg	Oral	Twice daily	7 days	After meals
Syrup LEVOLIN	3.5 mL	Oral	Every 8 hours	7 days	For airway relief



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DRUG NAME (GENERIC, CAPS)	DOSE	ROUTE	FREQUENCY	DURATION	INSTRUCTIONS
Syrup LAVERA (Levetiracetam)	2 mL	Oral	Every 12 hours	2 years	Continue without doses

☒ KPI: All medications in generic names, with dose, route, frequency, and duration clearly documented.

11. FOLLOW-UP INSTRUCTIONS

Parameter	Details
Next OPD Follow-up Date	04-Nov-2025 (After 5 days)
Specialty Review (If required)	Pediatric Neurology (for EEG follow-up)
Pending Investigations	None
Red Flag Signs	Seizure recurrence, fever, drowsiness, vomiting, breathing difficulty

Emergency Contact:

RADHAKISHAN HOSPITAL, Opp. New Bus Stand, Kurukshetra

Reception: 01744-312-067

Emergency: 01744-251441 / 270516 / 7206029516

☒ KPI: Follow-up and emergency contact provided.

12. PATIENT & FAMILY EDUCATION (BILINGUAL)

English:

- Continue all medicines exactly as prescribed.
- Lavera must be continued for 2 years—do not stop suddenly.
- Maintain hydration and nutrition.
- Keep child warm and avoid exposure to infection.
- Report to hospital if any fever, seizure, or respiratory difficulty occurs.

हिन्दी (Hindi):

- बच्चे को सभी दवाइयाँ समय पर दें, कोई खुराक न छोड़ें।
- लावेरा सिरप को 2 वर्ष तक लगातार देना आवश्यक है।
- बच्चे को पर्याप्त आहार और पर्याप्त पानी दें।
- ठंड या संक्रमण से बचाव करें।
- यदि बुखार, दौरा, या साँस लेने में तकलीफ हो तो तुरंत अस्पताल संपर्क करें।

☒ KPI: Family informed in bilingual format for compliance and safety.

13. IMMUNIZATION STATUS & PREVENTIVE CARE

Vaccine	Status	Next Due Date
Routine Immunization	Up to date	As per IAPCOI 2025 Schedule



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Date	Na ⁺ (mmol/L)	K ⁺ (mmol/L)	Ca ⁺⁺ (mmol/L)	Urea (mg/dL)	Creatinine (mg/dL)	RBS (mg/dL)	Remarks
22-Oct	136	3.4	4.6	22	0.61	104	Mild hypokalemia
24-Oct	138	3.2	4.8	19	0.58	96	Improving
26-Oct	137	3.0	4.7	17	0.53	96	Hypokalemia corrected
28-Oct	137	3.8	4.7	17	0.53	96	Normal
30-Oct	138	3.8	4.8	18	0.56	98	Normal at discharge

◆ 4 ABG MONITORING

Date	pH	pCO ₂ (mmHg)	pO ₂ (mmHg)	HCO ₃ ⁻ (Act)	HCO ₃ ⁻ (Std)	BE	Lactate (mmol/L)	FiO ₂	Remarks
22-Oct	7.356	39.1	74.2	20.0	21.0	-5.5	1.8	60%	Initial respiratory acidosis
24-Oct	7.365	33.4	72.7	18.7	19.5	-5.9	2.0	50%	Improving oxygenation
25-Oct	7.345	37.9	59.9	20.2	20.2	-5.0	1.29	40%	Stable post-extubation
26-Oct	7.463	30.5	107.6	21.4	23.0	-2.4	1.21	21%	Excellent gas exchange
28-Oct	7.441	32.1	98.4	22.0	23.1	-2.0	1.15	21%	Normal ABG off oxygen
30-Oct	-	-	-	-	-	-	-	21%	Not repeated (clinically stable)

◆ 5 MICROBIOLOGY & SPECIAL TESTS

Date	Test	Result	Remarks
23-Oct	Blood Culture	No Growth	Negative
24-Oct	CBNAAT (for TB)	MTB Not Detected	No tuberculosis
25-Oct	Typhidot IgM/IgG	Negative	-

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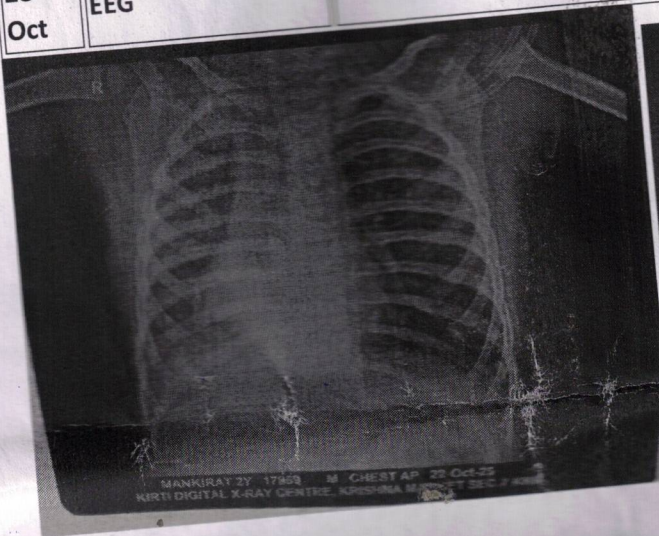
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Discharge Summary

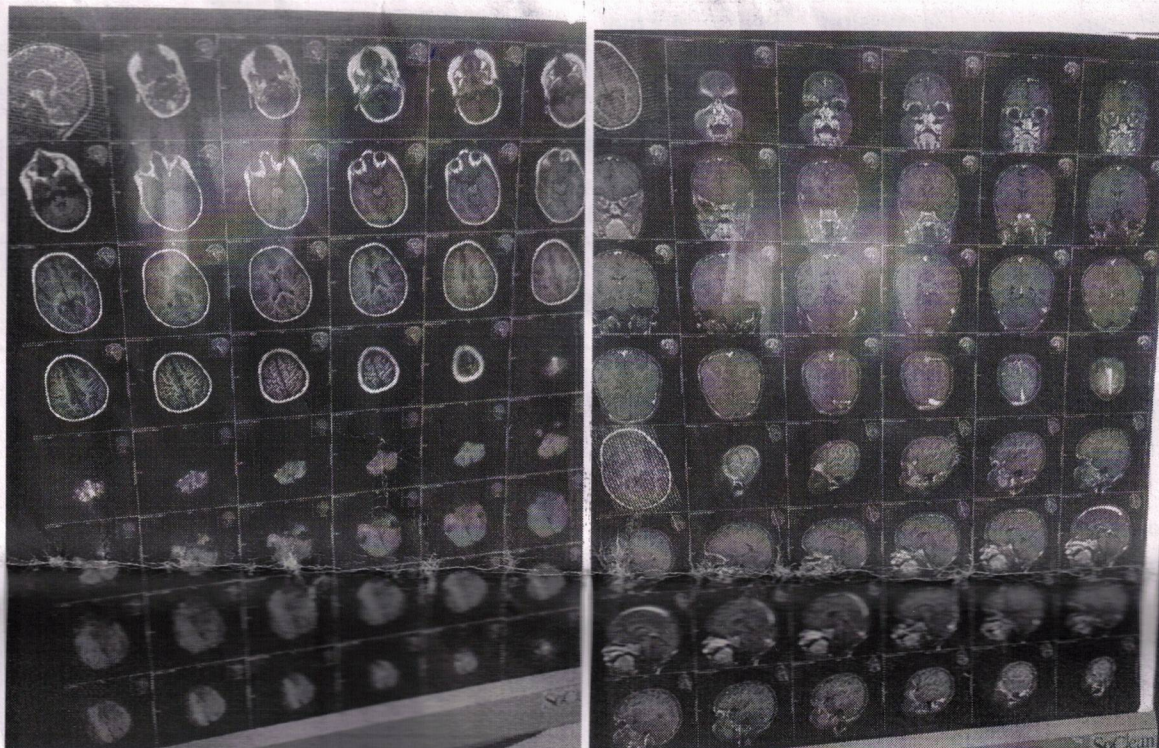
Date	Test	Result	Remarks
25-Oct	Widal	S. Typhi O 1:160, H 1:80	Reactive (past infection)
27-Oct	Urine R/M	Normal	—
28-Oct	EEG	Abnormal – Generalized Dysrhythmia	Post-infectious change
28-Oct	MRI Brain	Normal	No structural lesion

♦ 6 RADIOLOGY / IMAGING

Date	Investigation	Findings	Impression
22-Oct	Chest X-ray (AP View)	Right lower-zone opacity, volume loss	Right lower-lobe pneumonia
24-Oct	Chest X-ray (AP View)	Partial clearing of haziness	Improving consolidation
26-Oct	Chest X-ray (AP View)	Near-complete resolution	Resolving pneumonia
28-Oct	MRI Brain	Normal parenchyma, no meningeal enhancement	Normal MRI Brain
28-Oct	EEG	Generalized cerebral dysrhythmia	Post-septic encephalopathy pattern



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♦ 7 OTHER SCREENING / SUPPORTIVE TESTS

Date	Test	Result	Remarks
22-Oct	HIV, HBsAg, HCV	Negative	Screening protocol completed
23-Oct	LFT	Within normal limits	No hepatic dysfunction
25-Oct	Electrolyte Trend	Mild transient hypokalemia	Corrected
29-Oct	Physical Growth Chart	Weight 14 kg (stable)	No malnutrition
30-Oct	PEWS/PELOD-2	Score 0 / 0	Stable

♦ 8 SUMMARY OF INVESTIGATION TRENDS

Parameter	Admission	Peak Abnormal	At Discharge	Trend
CRP (mg/L)	56	56	2	↓ Improving
PCT (ng/mL)	0.78	0.78	0.04	↓ Normalized
TLC (/mm ³)	14,700	14,700	5,800	↓ Normalized
Hb (g/dL)	8.9	8.9	9.3	↑ Stable



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Parameter	Admission	Peak Abnormal	At Discharge	Trend
ESR (mm/hr)	—	—	8	Normal
pO ₂ (mmHg)	74	74	98	↑ Normal
Lactate (mmol/L)	1.8	1.8	1.15	↓ Normal
EEG / MRI	—	Dysrhythmia / Normal MRI	No change	Stable

☒ KPI: Serial improvement documented; infection resolved.

15. CONSULTANT & JUNIOR DOCTOR REVIEW

Consultant's Review & Signature:

PARENTS SIGNATURE:

Dr. Lokender Goyal, MD Pediatrics (HN21452)

Dr. Swati Singhal, MD, MRCPCH (HN2670)

Junior Doctor: Dr. Simran, MBBS - Hospital ID: RKH0380

Date of Discharge: 30-Oct-2025 | 4:25 PM

ANNEXURE 1 – For LAMA / DAMA / AMA Cases

Not applicable – routine discharge.

☒ ALL NABH HCO 6TH EDITION DISCHARGE SUMMARY KPI CHECKPOINTS MET

KPI Requirement	Included ✓	Section
Final Diagnosis with ICD-10	✓	Sec 3
Reason for Admission	✓	Sec 4
Clinical Course Summary	✓	Sec 6
Procedure Log	✓	Sec 7
Medications (In-hospital & Discharge)	✓	Sec 8–10
Condition at Discharge	✓	Sec 9
Follow-up Plan & Red Flags	✓	Sec 10
Family Education (Bilingual)	✓	Sec 12
Immunization Status	✓	Sec 13
Investigation Trends	✓	Sec 14
Emergency Contacts	✓	Sec 11

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KPI Requirement	Included ✓	Section
Consultant Signature Block	✓	Sec 15

Prepared & Authenticated By:
Consultant's Review & Signature:

PARENTS SIGNATURE:

Dr. Lokender Goyal, MD Pediatrics (HN21452)
Dr. Swati Singhal, MD, MRCPCH (HN2670)
Junior Doctor: Dr. Simran, MBBS - Hospital ID: RKH0380
(Signature & Seal – 30-Oct-2025, 2:25 PM)